

- CT BRAIN WO CONTRAST Date: 2/8/2015

REASON FOR EXAM: TBI

COMPARISON: CT brain 2/7/2015

Technique: Helical axial tomographic images of the brain were obtained without IV contrast.

Findings:

The tiny subdural hematoma overlying the right frontal lobe is not clearly seen. There appears to be a minimal amount of subdural hemorrhage along both sides the posterior falx and layering over the right tentorium. Midline structures are intact and nondisplaced. No evidence of mass effect or edema. Gray-white differentiation is maintained. Ventricles and cisterns are normal in size and configuration. The basal ganglia and posterior fossa are unremarkable.

The calvarium is intact. There is mild mucosal thickening in the visualized paranasal sinuses. The mastoid air cells are clear. The visualized intraorbital contents are grossly maintained.

Impression:

Minimal subdural hemorrhage along the posterior falx and right tentorium.

** Electronically Signed by M.D. SSR 280 WAYNE KISHIMOTO **

** on 02/08/2015 at 0811 **

Reported and signed by: WAYNE KISHIMOTO, M.D. SSR 280



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15.2.08_MRI_OKC.PNG



- MR BRAIN W WO INF 004324441 Date: 2/8/2015 1:30 PM

ADMIT DX: PT263 MVC, STERNAL FX, HUMERUS FX, PULM CONTUSION

REASON FOR EXAM: TBI

COMPARISON: CT brain 2/8/2015

TECHNIQUE: Using a 1.5 tesla magnet, multiplanar magnetic resonance imaging was performed of the brain before and after the administration of 15 cc of ProHance contrast material without complication.

Findings:

Foci of restricted diffusion within the left caudate and right subinsular cortex demonstrates increased intensity on the eADC map suggesting T2 shine through. Susceptibility gradient and FLAIR hyperintensity are demonstrated within these foci. In addition, susceptibility artifact is demonstrated at the gray-white interface within the bifrontal lobes. The midline structures are nondisplaced. The gray-white differentiation is maintained. The ventricular system is appropriate in size and configuration with no evidence of hydrocephalus. No intraparenchymal hemorrhage, mass-effect, or abnormal extra-axial fluid collection is present. The basilar cisterns are preserved. There is no abnormal post contrast enhancement.//

A flow void is present in the major intracranial arterial and dural venous structures suggesting patency by T2 spin echo criteria. The regions of the dural venous sinuses, meninges, sella, and orbits are normal in signal intensity on all pulse sequences. The paranasal sinuses, mastoid air cells, and middle ear cavities are low in signal intensity on all pulse sequences.

Impression:

Multifocal susceptibility artifact at the gray-white junction may represent sequela of DAI. T2/FLAIR hyperintensity within the left caudate and right subinsular cortex may represent edema.

79.6 KB



85%

